Patient Record — ID 1709

Age	34	**Sex**	F
Race/Ethnicity	African American	**Insurance**	Private insurance
Total Encounters	8	**Generation Seed**	3685

Primary Diagnoses

• Placenta previa with preterm hemorrhage
• Idiopathic polyhydramnios
• Acute fatty liver of pregnancy
• Urethral diverticulum
• Breech presentation at 36 weeks gestation
• Transverse lie
• History of classical cesarean section
• Severe preeclampsia

Comorbidities

• Personal History of Cholelithiasis
• Multiparity
• Pregnancy at 36 weeks gestation

Patient Profile

Sex	F
Age at First Encounter	34
Race/Ethnicity	African American
Insurance	Private insurance
Occupation	Elementary school teacher
Smoking Status	Never smoker
Alcohol Use	No alcohol use

Allergies:

• No known drug allergies

Chronic Conditions:

• Uterine leiomyomas
• Urethral diverticulum
• History of gestational hypertension

Home Medications:

• Prenatal vitamin with iron - 1 tablet daily
• Low-dose aspirin - 81 mg daily
• Nitrofurantoin - 100 mg nightly for UTI prophylaxis

Surgical History:

• Classical cesarean section (age 32)
• Repair of second-degree perineal tear (age 30)

Family History:

• Preeclampsia (mother)
• Uterine leiomyomas (mother)
• Hypertension (father)

Encounters (8)

Encounter 1 — 2020-01-15 — Obstetrics & Gynecology

Encounter ID	7019	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	0
Department	Obstetrics & Gynecology	**Gen. Method**	template

Chief Complaint:

recurrent urinary tract infections, dysuria, and a sensation of incomplete bladder emptying

Full Note:

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ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Recurrent urinary tract infections, dysuria, and a sensation of incomplete bladder emptying

HISTORY OF PRESENT ILLNESS:
A 32-year-old woman presents to the clinic with a 6-month history of recurrent urinary tract
infections, dysuria, and a sensation of incomplete bladder emptying. She also reports occasional
postvoid dribbling and dyspareunia.

PAST MEDICAL HISTORY:
Active Problem List:
- Uterine leiomyomas
- Urethral diverticulum
- History of gestational hypertension

Her past medical history is significant for childbirth 2 years ago with a second-degree perineal
tear.

PAST SURGICAL HISTORY:
- Classical cesarean section (age 32)
- Repair of second-degree perineal tear (age 30)

MEDICATIONS:
Home Medications:
- Prenatal vitamin with iron 1 tablet daily
- Low-dose aspirin 81 mg daily
- Nitrofurantoin 100 mg nightly for UTI prophylaxis

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
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- Preeclampsia (mother)
- Uterine leiomyomas (mother)
- Hypertension (father)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: No alcohol use. Occupation: Elementary school teacher.

PHYSICAL EXAMINATION:
On physical examination, a tender anterior vaginal wall mass is palpated, which expresses purulent discharge when pressure is applied

LABORATORY DATA:
Urinalysis reveals pyuria

Encounter 2 — 2020-03-15 — Obstetrics & Gynecology

Encounter ID	7017	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	1
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

presents to the obstetrics clinic for a routine prenatal visit

Full Note:

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ENCOUNTER NOTE
Date: 2020-03-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
She presents to the obstetrics clinic for a routine prenatal visit

HISTORY OF PRESENT ILLNESS:
The patient returns for routine prenatal care with uterine leiomyomas, urethral diverticulum, and
gestational hypertension. She reports feeling well with no complaints of contractions, vaginal
bleeding, or leakage of fluid, and her pregnancy has been uncomplicated with regular prenatal
care.

PAST MEDICAL HISTORY:
Active Problem List:
- Urethral diverticulum (diagnosed 2020-01-15)
- Uterine leiomyomas
- History of gestational hypertension

Her first pregnancy resulted in a normal vaginal delivery of a healthy infant.

PAST SURGICAL HISTORY:
- Classical cesarean section (age 32)
- Repair of second-degree perineal tear (age 30)

MEDICATIONS:
Home Medications:
- Prenatal vitamin with iron 1 tablet daily
- Low-dose aspirin 81 mg daily
- Nitrofurantoin 100 mg nightly for UTI prophylaxis

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Preeclampsia (mother)
- Uterine leiomyomas (mother)
- Hypertension (father)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: No alcohol use. Occupation: Elementary school teacher.

VITAL SIGNS:
On examination, her vital signs are within normal limits

PHYSICAL EXAMINATION:
Fetal heart tones are regular. The fundal height measurement is noted to be greater than expected
for gestational age.
```

IMAGING:
An ultrasound is performed, which reveals an amniotic fluid index (AFI) of 26 cm with no fetal anomalies or evidence of gastrointestinal obstruction. The fetus is in a cephalic presentation with normal growth parameters.

Encounter 3 — 2020-06-15 — Obstetrics & Gynecology

Encounter ID	7021	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	2
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

presents to the obstetrics clinic for a routine prenatal visit

Full Note:

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ENCOUNTER NOTE
Date: 2020-06-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
She presents to the obstetrics clinic for a routine prenatal visit

HISTORY OF PRESENT ILLNESS:
Patient returns for prenatal care with leiomyomas and diverticulum and reports feeling well with
good fetal movement. Prenatal course is unremarkable.

PAST MEDICAL HISTORY:
Active Problem List:
- Urethral diverticulum (diagnosed 2020-01-15)
- Idiopathic polyhydramnios (diagnosed 2020-03-15)
- Uterine leiomyomas
- History of gestational hypertension

Her first pregnancy was uncomplicated, and she had a vaginal delivery at term.

PAST SURGICAL HISTORY:
- Classical cesarean section (age 32)
- Repair of second-degree perineal tear (age 30)

MEDICATIONS:
Home Medications:
- Prenatal vitamin with iron 1 tablet daily
- Low-dose aspirin 81 mg daily
- Nitrofurantoin 100 mg nightly for UTI prophylaxis

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Preeclampsia (mother)
- Uterine leiomyomas (mother)
- Hypertension (father)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: No alcohol use. Occupation: Elementary school teacher.

VITAL SIGNS:
Her blood pressure is 120/78 mmHg, heart rate is 82 beats per minute

PHYSICAL EXAMINATION:
On examination, fundal height is consistent with gestational age. Fetal heart tones are normal.

IMAGING:
```

An ultrasound is performed, which shows the fetus in a transverse lie with the spine perpendicular to the mother's long axis. There are no other abnormalities noted on the ultrasound.

Encounter 4 — 2020-08-15 — Obstetrics & Gynecology

Encounter ID	7020	**Type**	outpatient
Attending	Dr. Maria Rodriguez	**Order**	3
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

presents to the obstetrics clinic for a routine prenatal visit

Full Note:

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ENCOUNTER NOTE
Date: 2020-08-15
Provider: Dr. Maria Rodriguez, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
She presents to the obstetrics clinic for a routine prenatal visit

HISTORY OF PRESENT ILLNESS:
She returns for prenatal care at 36 weeks with diverticulum and transverse lie. She reports
feeling well but notes fetal movements differ from first pregnancy.

PAST MEDICAL HISTORY:
Active Problem List:
- Urethral diverticulum (diagnosed 2020-01-15)
- Idiopathic polyhydramnios (diagnosed 2020-03-15)
- Transverse lie (diagnosed 2020-06-15)
- Uterine leiomyomas
- History of gestational hypertension

Her past medical history is significant for uterine leiomyomas diagnosed two years ago, which have
been asymptomatic and monitored with serial ultrasounds. Her first child was delivered vaginally
at term without complications.

PAST SURGICAL HISTORY:
- Classical cesarean section (age 32)
- Repair of second-degree perineal tear (age 30)

MEDICATIONS:
Home Medications:
- Prenatal vitamin with iron 1 tablet daily
- Low-dose aspirin 81 mg daily
- Nitrofurantoin 100 mg nightly for UTI prophylaxis

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Preeclampsia (mother)
- Uterine leiomyomas (mother)
- Hypertension (father)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: No alcohol use. Occupation: Elementary school teacher.

PHYSICAL EXAMINATION:
On examination, the fundal height is appropriate for gestational age, and fetal heart tones are
normal.
```

IMAGING:
An ultrasound is performed, which shows the fetus in a breech position with the buttocks presenting closest to the cervix. The ultrasound also confirms the presence of multiple leiomyomas, the largest of which is located at the fundus of the uterus.

Encounter 5 — 2020-09-15 — Obstetrics & Gynecology

Encounter ID	7022	**Type**	inpatient
Attending	Dr. Maria Rodriguez	**Order**	4
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

presents to the labor and delivery unit with regular contractions every 5 minutes for the past 2 hours

Full Note:

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ENCOUNTER NOTE
Date: 2020-09-15
Provider: Dr. Maria Rodriguez, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
She presents to the labor and delivery unit with regular contractions every 5 minutes for the past
2 hours

HISTORY OF PRESENT ILLNESS:
Returning to the labor and delivery unit with regular contractions every 5 minutes, the patient
has histories of uterine leiomyomas, urethral diverticulum, and gestational hypertension, and was
evaluated earlier this pregnancy for recurrent urinary symptoms, polyhydramnios, and fetal
malpresentation. Her prenatal course was unremarkable, and she desires a vaginal delivery.

PAST MEDICAL HISTORY:
Active Problem List:
- Urethral diverticulum (diagnosed 2020-01-15)
- Idiopathic polyhydramnios (diagnosed 2020-03-15)
- Transverse lie (diagnosed 2020-06-15)
- Breech presentation at 36 weeks gestation (diagnosed 2020-08-15)
- Uterine leiomyomas
- History of gestational hypertension

PAST SURGICAL HISTORY:
She reports that her previous delivery was via cesarean section due to breech presentation. On
further questioning, she reveals that the cesarean was performed through a vertical incision on
the upper uterus because the baby was in an awkward position.

MEDICATIONS:
Home Medications:
- Prenatal vitamin with iron 1 tablet daily
- Low-dose aspirin 81 mg daily
- Nitrofurantoin 100 mg nightly for UTI prophylaxis

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Preeclampsia (mother)
- Uterine leiomyomas (mother)
- Hypertension (father)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: No alcohol use. Occupation: Elementary school teacher.

VITAL SIGNS:
On examination, her vital signs are stable
```

PHYSICAL EXAMINATION:
Fetal heart tones are reassuring. The cervix is dilated to 4 cm, and the fetus is in a cephalic presentation.

Encounter 6 — 2023-01-15 — Obstetrics & Gynecology

Encounter ID	7016	**Type**	outpatient
Attending	Dr. James Okafor	**Order**	5
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

presents to the obstetrics clinic for a routine prenatal visit.

Full Note:

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ENCOUNTER NOTE
Date: 2023-01-15
Provider: Dr. James Okafor, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
She presents to the obstetrics clinic for a routine prenatal visit.

HISTORY OF PRESENT ILLNESS:
Patient with uterine leiomyomas and prior cesarean returns for prenatal care. Her current
pregnancy has been unremarkable until now. She reports occasional shortness of breath but denies
vaginal bleeding, leakage of fluid, or contractions.

PAST MEDICAL HISTORY:
Active Problem List:
- Urethral diverticulum (diagnosed 2020-01-15)
- Idiopathic polyhydramnios (diagnosed 2020-03-15)
- Transverse lie (diagnosed 2020-06-15)
- Breech presentation at 36 weeks gestation (diagnosed 2020-08-15)
- History of classical cesarean section (diagnosed 2020-09-15)
- Uterine leiomyomas
- History of gestational hypertension

Her first pregnancy was uncomplicated, and she had a spontaneous vaginal delivery at term.

PAST SURGICAL HISTORY:
- Classical cesarean section (age 32)
- Repair of second-degree perineal tear (age 30)

MEDICATIONS:
Home Medications:
- Prenatal vitamin with iron 1 tablet daily
- Low-dose aspirin 81 mg daily
- Nitrofurantoin 100 mg nightly for UTI prophylaxis

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Preeclampsia (mother)
- Uterine leiomyomas (mother)
- Hypertension (father)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: No alcohol use. Occupation: Elementary school teacher.

VITAL SIGNS:
On examination, her blood pressure is 120/78 mmHg, heart rate is 88 beats per minute, and fetal
heart rate is 140 beats per minute.
```

PHYSICAL EXAMINATION:
The obstetric examination reveals a fundal height consistent with gestational age, but the fetal position is suspected to be breech on palpation.

LABORATORY DATA:
Her prenatal labs have been normal, including a negative gestational diabetes screen.

IMAGING:
An ultrasound confirms a breech presentation with the fetus in a frank breech position. Additionally, the ultrasound shows placenta previa with the placenta covering the internal cervical os.

Encounter 7 — 2023-07-15 — Emergency Medicine

Encounter ID	7023	**Type**	ed
Attending	Dr. Emily Watson	**Order**	6
Department	Emergency Medicine	**Gen. Method**	hybrid

Chief Complaint:

severe right upper quadrant pain, nausea, and vomiting

Full Note:

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ENCOUNTER NOTE
Date: 2023-07-15
Provider: Dr. Emily Watson, MD
Department: Emergency Medicine
Visit Type: ed

CHIEF COMPLAINT:
Severe right upper quadrant pain, nausea, and vomiting

HISTORY OF PRESENT ILLNESS:
This 37-year-old female with uterine leiomyomas, urethral diverticulum on UTI prophylaxis, and
placenta previa with hemorrhage presents to the emergency department with a 24-hour history of
severe right upper quadrant pain, nausea, and vomiting. She reports the pain is constant and
radiates to her back.

PAST MEDICAL HISTORY:
Active Problem List:
- Urethral diverticulum (diagnosed 2020-01-15)
- Idiopathic polyhydramnios (diagnosed 2020-03-15)
- Transverse lie (diagnosed 2020-06-15)
- Breech presentation at 36 weeks gestation (diagnosed 2020-08-15)
- History of classical cesarean section (diagnosed 2020-09-15)
- Placenta previa with preterm hemorrhage (diagnosed 2023-01-15)
- Uterine leiomyomas
- History of gestational hypertension

She has a history of gestational hypertension in her previous pregnancy.

PAST SURGICAL HISTORY:
- Classical cesarean section (age 32)
- Repair of second-degree perineal tear (age 30)

MEDICATIONS:
Home Medications:
- Prenatal vitamin with iron 1 tablet daily
- Low-dose aspirin 81 mg daily
- Nitrofurantoin 100 mg nightly for UTI prophylaxis

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Preeclampsia (mother)
- Uterine leiomyomas (mother)
- Hypertension (father)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: No alcohol use. Occupation: Elementary school teacher.

VITAL SIGNS:
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Her blood pressure is 160/100 mmHg, and her pulse is 98 beats per minute.

PHYSICAL EXAMINATION:
Physical examination reveals right upper quadrant tenderness without rebound or guarding. Reflexes are brisk, and there is no ankle edema.

LABORATORY DATA:
Laboratory studies show:

- Hemoglobin: 9.8 g/dL (Normal: 12-16 g/dL)
- Platelet count: 70,000/mm³ (Normal: 150,000-400,000/mm³)
- AST: 120 U/L (Normal: 10-40 U/L)
- ALT: 150 U/L (Normal: 7-56 U/L)
- LDH: 600 U/L (Normal: 140-280 U/L)
- Total bilirubin: 2.2 mg/dL (Normal: 0.1-1.2 mg/dL)
- Peripheral smear: schistocytes present

Urinalysis shows 2+ proteinuria.

OTHER STUDIES:
The fetal heart tracing is reactive without decelerations.

Encounter 8 — 2024-07-15 — Emergency Medicine

Encounter ID	7018	**Type**	ed
Attending	Dr. David Kim	**Order**	7
Department	Emergency Medicine	**Gen. Method**	hybrid

Chief Complaint:

presents to the emergency department with a 2-day history of persistent nausea, vomiting, and right upper quadrant pain

Full Note:

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ENCOUNTER NOTE
Date: 2024-07-15
Provider: Dr. David Kim, MD
Department: Emergency Medicine
Visit Type: ed

CHIEF COMPLAINT:
She presents to the emergency department with a 2-day history of persistent nausea, vomiting, and
right upper quadrant pain

HISTORY OF PRESENT ILLNESS:
The patient with uterine leiomyomas, urethral diverticulum, and prior severe preeclampsia returns
with similar complaints. She reports that her pain is constant and radiates to her back. She
denies any history of gallstones or alcohol use. Her pregnancy has been uncomplicated until now.

PAST MEDICAL HISTORY:
Active Problem List:
- Urethral diverticulum (diagnosed 2020-01-15)
- Idiopathic polyhydramnios (diagnosed 2020-03-15)
- Transverse lie (diagnosed 2020-06-15)
- Breech presentation at 36 weeks gestation (diagnosed 2020-08-15)
- History of classical cesarean section (diagnosed 2020-09-15)
- Placenta previa with preterm hemorrhage (diagnosed 2023-01-15)
- Severe preeclampsia (diagnosed 2023-07-15)
- Uterine leiomyomas
- History of gestational hypertension

PAST SURGICAL HISTORY:
- Classical cesarean section (age 32)
- Repair of second-degree perineal tear (age 30)

MEDICATIONS:
Home Medications:
- Prenatal vitamin with iron 1 tablet daily
- Low-dose aspirin 81 mg daily
- Nitrofurantoin 100 mg nightly for UTI prophylaxis

ALLERGIES:
No known drug allergies

FAMILY HISTORY:
- Preeclampsia (mother)
- Uterine leiomyomas (mother)
- Hypertension (father)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: No alcohol use. Occupation: Elementary school teacher.
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VITAL SIGNS:
On examination, her blood pressure is 135/85 mmHg, heart rate is 102 beats per minute, and she is afebrile.

PHYSICAL EXAMINATION:
Her skin and sclera are anicteric. Abdominal examination reveals tenderness in the right upper quadrant without rebound or guarding. Fetal heart tones are normal.

LABORATORY DATA:
Laboratory studies reveal:

- Hemoglobin: 11.2 g/dL
- Platelet count: 85,000/mm^3
- Aspartate aminotransferase (AST): 250 U/L
- Alanine aminotransferase (ALT): 230 U/L
- Bilirubin: 1.2 mg/dL
- Alkaline phosphatase: 120 U/L
- Prothrombin time (PT): 16 seconds (control 12 seconds)
- Blood glucose: 45 mg/dL

Urinalysis is normal. There is no proteinuria.

PLAN:
The obstetric team is consulted.